

Methadone	Buprenorphine
PD: Full agonist	PD: Partial agonist
PK: Long lasting, ½ life around 7 days	PK: Long acting dosed around 2-3x/wk outpt
SE: QTC prolongation and resp depression	SE: Precipitate withdrawal, minimal resp depression
Info: #We can start in hospital with no issues #Daily clinic for first couple weeks #Cheaper (approx: 100/month) #Wait list of 2 weeks+, or so #Not good for QT prolongation patients #bad part of town, accosted in line by drug dealers #Use if pt still needs opioids in hospital #will need to return to same clinic and need transportation to clinic #typical session is: wait in line→ urine test→ counseling→ receive→ methadone	Info: #We can start in hospital with no issues #Do not use if patient still going to need opioids for pain #Expensive, will need insurance #Will need to find provider to prescribe #Dosing can be weekly or up to monthly #Good if concerned about QT prolongation
Dosing: #Minimal restrictions or concerns for starting in hosp. #monitor for COWS withdrawal #1 Start 5-10mg Wait 1-2 hours #2 Add another 5-10mg Wait 2-4 hours #3 Add 5-10mg Repeat until symptoms resolve, avoid sedating pt #Total daily dose used is the Daily dose needed #May administer as QD or TID if for pain #Should not exceed 30-40mg for most patients #<u>Max duration</u> is up to 3 days of meds Post Discharge, prior to needing outpatient script #If converting from morphine is tiered by how much morphine pt was on. 10-30Morphine: 50% (5-15mg methadone) 30-100M : 25%-50% 100-200M: 15-30% 200-500: 10-20%	Dosing: #Determine need for future opioid use #Determine insurance/monetary needs #Will need to monitor for withdrawal prior to administration (Tachy, HTN, incr Resp, yawing, mydriasis, Incr Bowel sounds, diaphoresis) #Start 2-4mg (4 more common) Wait 1-2 hours #Add 2-4 mg #Total Daily Dose -8mg or 8mg BID - can go up to 32 mg daily but 16 more common #<u>Max duration</u> is up to 3 days of meds Post Discharge, prior to needing outpatient script Suboxone: Buprenorphine/Naloxone combo pill -Combo pill can have bitter taste to some -2/.05mg, 4/1mg, 8/2mg -Sublingual or tablet

Opiate withdrawal protocol:

#Clonidine }
#Zofran }Schedule these
#NSAID }

#Dicyclomine)
#Methocarbamol) PRN these
#Ativan)
#Loperamide)

Suboxone: buprenorphine + Naloxone